Information For the Use of
Furosemide (20mg) + Spironolactone (50mg) Tablets

Furotide S Tablet

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Composition:

Each tablet contains:

- **Furosemide** – 20 mg
- **Spironolactone** – 50 mg

Pharmacological Action:

Furotide S combines a **loop diuretic** with a **potassium-sparing diuretic and aldosterone antagonist** to promote sodium and water excretion while helping reduce potassium loss.

- **Furosemide:** Inhibits sodium and chloride reabsorption in the thick ascending limb of the loop of Henle, producing potent diuresis and reducing fluid overload.
- **Spironolactone:** Blocks aldosterone receptors in the distal nephron, promoting sodium and water excretion while reducing potassium and hydrogen ion loss.
- **Combined Action:** Produces complementary diuretic effects and helps manage **fluid retention and edema** in appropriate patients.
- **Reduced Potassium Loss:** Spironolactone may counteract the potassium-lowering effect of furosemide, although serum potassium must still be monitored.

Indications:

Furotide S Tablet is indicated for the management of fluid retention associated with:

- **Congestive Heart Failure**
- **Hepatic Cirrhosis with Edema or Ascites**
- **Edema associated with Renal Disorders**, when clinically appropriate
- **Secondary Hyperaldosteronism**
- Other conditions with clinically significant fluid retention, as prescribed by the physician

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Dosage and Administration:

- **Recommended Dose:** Dosage should be individualized according to the patient's clinical condition, response, renal function, and electrolyte status.
- Take preferably **in the morning**, with or without food, as directed by the physician.

- If a second daily dose is prescribed, it is generally taken in the early afternoon to minimize nighttime urination.
- Swallow the tablet whole with water.
- Maintain adequate fluid intake according to the physician's advice.
- Regular monitoring of **blood pressure, renal function, serum electrolytes, and body weight** may be required.
- Do not discontinue or alter the dose without medical advice.

Precautions:

- Monitor **serum potassium, sodium, creatinine, and renal function** regularly.
- Spironolactone may cause **hyperkalemia**, particularly in patients with renal impairment or those receiving other potassium-raising medicines.
- Furosemide may cause **dehydration, hypotension, hyponatremia, hypokalemia, and other electrolyte disturbances**.
- Use cautiously in patients with **kidney disease, liver disease, diabetes, gout, or low blood pressure**.
- Rapid or excessive diuresis in patients with cirrhosis may cause complications, including renal impairment and electrolyte disturbances.
- Avoid potassium supplements or potassium-containing salt substitutes unless specifically prescribed.
- Prolonged or high-dose furosemide therapy may rarely cause **ototoxicity**, particularly with rapid parenteral administration or concomitant ototoxic medicines.
- **Pregnancy and Lactation:** Use only when clearly indicated and under medical supervision.

Contraindications:

- Known hypersensitivity to **furosemide, spironolactone, sulfonamide-related compounds, or any component of the formulation**.
- **Hyperkalemia**
- **Severe renal impairment with anuria**
- **Addison's disease**
- Severe electrolyte depletion until corrected
- Concomitant use with **potassium supplements or potassium-sparing agents** may be contraindicated or require specialist supervision depending on the clinical situation.

Adverse Reactions:

Common:

- Increased urination
- Dizziness
- Headache

- Dehydration
- Low blood pressure
- Electrolyte disturbances
- Nausea or gastrointestinal discomfort

Spironolactone-related:

- Hyperkalemia
- Breast tenderness
- Gynecomastia
- Menstrual irregularities
- Fatigue

Less Common but Serious:

- Severe dehydration or hypotension
- Acute kidney injury
- Significant electrolyte imbalance
- Severe hyperkalemia
- Hearing impairment or tinnitus
- Serious hypersensitivity reactions

Seek medical attention for severe weakness, confusion, fainting, irregular heartbeat, markedly reduced urine output, or severe dehydration.

Drug Interactions:

- **ACE inhibitors and ARBs** may significantly increase the risk of hyperkalemia and renal impairment.
- **Potassium supplements and potassium-containing salt substitutes** may increase the risk of hyperkalemia.
- **Other potassium-sparing diuretics** should generally not be combined unless specifically prescribed.
- **NSAIDs**, including ibuprofen and naproxen, may reduce diuretic efficacy and increase the risk of renal impairment and hyperkalemia.
- **Digoxin:** Electrolyte disturbances may increase the risk of digoxin toxicity.
- **Lithium:** Furosemide may reduce lithium clearance and increase lithium toxicity.
- **Corticosteroids** and other medicines affecting electrolytes may increase the risk of electrolyte abnormalities.
- **Other antihypertensive medicines** may produce additive hypotensive effects.
- Inform the physician about all prescription medicines, OTC products, and supplements being used.

Storage:

- Store in a cool, dry place below **25°C**.
- Protect from moisture, direct sunlight, and excessive heat.
- Keep tablets in the original packaging.
- Keep out of reach of children.

Note:

This information is intended as a general prescribing guide. **Furotide S Tablet should be used under the supervision of a registered medical practitioner**, particularly in patients with heart failure, liver disease, or renal impairment.

Important: Regular monitoring of **serum potassium and renal function** is important because the combination can cause either potassium depletion or potassium elevation depending on the patient's condition and concomitant medicines. Avoid self-medication with potassium supplements or potassium-containing salt substitutes unless specifically advised by the physician.

Manufactured in India for:

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